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HAVEN PAEDIATRIC CENTRE

The Town Hall

Speaking notes for the Haven staff address

SPEAKER
Dr Debo Odulana, Founding Partner

AUDIENCE
Haven Paediatric Centre, all staff

DATE
6 October 2026

CIRCULATION
Internal to Consult for Africa. Your card, not a handout.

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Before you stand up

Three rules for this room.

Do not mention May. Not obliquely, not as "what brought us here", not in answer to a question. If somebody raises it, say you are not going to discuss any individual patient in a room like this, thank them, and move on. Nothing you could say helps, and everything you say travels.

No numbers about money. Not the fee, not the concession, not the receivables, not what the hospital bills. Staff hear revenue figures as a conversation about their salary, and you will spend the rest of the hour on a subject you did not come to discuss.

Do not promise pay. Not a rise, not a bonus, not a timeline. You can say the incentive and job description work is happening and that it is being designed to reward good care rather than activity. That is true and it is enough.

Do not promise jobs either. The plan outsources routine laboratory work, and Linda Nwabuisi is the only Medical Lab Scientist on the staff list. She will be in the room. A flat "nobody is losing their job" is a promise you cannot keep, and a room remembers that for years. The wording in section 8 holds without it.

Do not introduce Dr Gbadebo, and do not use the words head of operations about anybody. Benedict Ijehon already holds admin, operations and business development, and he is in the room. If she is on site or comes up at all, she is a project consultant and lead from Consult for Africa, working on the engagement. That is the whole answer. Her future role is not a town hall subject and will not be one until Benedict's position has been settled with him privately.

01 Who is in the room

Nineteen people. Small enough that naming individuals works and not naming them is noticed.

Department	Who
Medical	Dr Adegbajo Odedina (Chief Medical Director, and a founder), Eniola Akinnuoye, Amarachi Onwuegbuchulam, Juliet Olajesu (Medical Officers)
Admin	Benedict Ijehon (Head of Admin, Operations and BD), Toyin Asagidingbi (Head of Customer Service), Moshood Saliu (Head of Finance), Francis Kadiri (Admin and Billing Officer)
Nursing	Chioma Agomuo (Senior Registered Nurse), Margaret Alada, Waliyat Aloko, Anita Musa, Okorie Sandra, Munachi Chiamara
Lab	Linda Nwabuisi (Medical Lab Scientist)
Pharmacy	Blessing Onuche, Janet Ogah (Pharmacy Technicians)
Domestic	Kosichukwu Ubanwa, Cynthia Eziwanne (Attendants)

Three people to be deliberate about. Dr Odedina is both the Chief Medical Director and an owner, so anything you say about the board he hears twice. Benedict Ijehon carries admin, operations and business

development on his own, which is three jobs, and he should hear you say that out loud in front of his team. Francis Kadiri is the billing officer, which makes him the single most important person to the tariff and capture work, and he is likely to assume the finding about unbilled work is about him. It is not. Say so, by name if you can.

Worth knowing privately: six nurses cover five paediatric beds and three NICU cots around the clock. That is not enough for a rota that survives leave and sickness, and it is the establishment question in the handbook made real. Not a town hall subject.

02 Open on them, not on you

Lead with their own survey. It is anonymous, it is theirs, and it is the single best thing you can put in that room.

> Sixteen of you out of seventeen rated the safety of care here as very good or excellent. You said that anonymously, when you could have said anything. I have worked with a lot of hospitals and that result is rare, and it is not an accident. It is the thing I most want to protect.

Then say what that means: the hardest part of building a hospital is already done here, and it was done by them, before anyone like you turned up.

03 Why we are here

Forward-looking, every word.

> Haven is a little over a year old and it is growing. What we are here to do is put proper systems underneath a hospital that has been held together by people caring enough to make it work. That is not a criticism. It is what happens in every young hospital that is any good, and the ones that do not do this part are the ones that stop being good when they get busy.

The line to use, because it is ours and it is true: culture is the multiplier. Everything else works better because of what they already have.

04 What we found, said so nobody hears blame

Say it as a system finding, never a performance finding.

> We found that the procedures here exist and are good, and that almost nobody has been trained on them. Supervision is informal and nobody checks. That is not anybody in this room failing. If you were never shown it, you cannot be held to it, and that is on the people running the place rather than on you.

Three things you can name safely because they are about systems and not about people: the way work gets recorded and billed, the way stock is ordered so the shelf is not empty, and the fact that the

procedures that exist have never been trained out or checked, so care depends on who is on shift.

05 What changes, and what is in it for them

This is the part they came for. Be concrete.

What changes	What it means on their shift
Standards of work, written with them	You stop being blamed for doing it the way you were taught
Stock and reordering fixed	The drug is on the shelf when you reach for it
Roster published four weeks ahead	You can plan your life
A proper handover and a daily huddle	You find out what happened before you walk into it
Recording and billing fixed	Less paperwork chasing, not more
Incentives and job descriptions reworked	What good looks like is written down, and it rewards care

Say plainly that the standards will be written with them and not at them. The sentence that works: show me how you do this now.

06 The season

> From November to February this hospital gets busier than at any other time of year. Everything we are doing now is so that you go into that season with the stock, the rota and the standards already in place, rather than finding out in December.

That reframes the pace as protection rather than pressure, which is the honest version anyway.

07 The ask

One ask only, or it will not land.

> When something nearly goes wrong and you catch it, I want to know. Not so anybody gets into trouble. A near miss is the cheapest lesson a hospital ever gets, and the hospitals that learn from them are the ones that stay safe when they get busy. If reporting one ever costs somebody here anything, that is a failure of mine and I want to hear about it.

> *Close on this.*
>
> *You built the part that cannot be bought. We are here to build the part that can, so that the thing you built survives the hospital getting bigger. Tell us what is broken. We will be in the building.*

08 Questions you will get

They ask	You say
Are people losing their jobs?	Nobody's job is being removed because of anything we found. The findings are about systems and forms, not about people. Where a service changes shape I will say so to the people affected first, not in a room.
Is the lab being outsourced?	Some routine testing will move to a reference partner so results come back faster and the hospital stops carrying the cost of running everything in house. The point of care testing stays here and so does the work of running it. If that affects anybody specifically, I will talk to them directly.
Are salaries going up?	That is the board's decision and not mine to announce. What I can tell you is the incentive and job description work is happening and it is being built to reward good care.
Why is a consultant here?	Because growing well is a different skill from running well, and it is cheaper to borrow it than to learn it in public.
Who is the new person I saw?	A project consultant and lead from our side, working on the engagement with us. You will meet the team properly as the work gets going.
Is somebody being blamed?	No. We diagnose a system, never a person. Nothing we have found is about anybody's performance.
Will it be more paperwork?	Less. Most of what we are changing is to stop you writing the same thing three times.
What about the May situation?	I am not going to discuss any individual patient in a room like this.

09 Last check before you walk in

- No mention of May, in any form
- No money figures of any kind
- No promise about pay
- Opened on their survey result, not on your findings
- Said nothing about Dr Gbadebo beyond "a project consultant and lead from our side"
- Did not use the words head of operations about anybody
- Credited Benedict out loud for carrying three jobs
- Said nothing that promises Linda's lab role is unchanged
- Told Francis the billing finding is not about him
- Asked for near misses, and said nobody gets in trouble